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Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
PRESIDENTIAL EBOLA TASK FORCE 17
MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°153 /MVE-BDBV/14/10/2026

Report date: October 14, 2026 Publication date: October 15, 2026

HIGHLIGHTS (last 24 hours)

- For October 14, 2026, 65 new confirmed cases of Ebola Virus Disease (EVD) were reported, including 22 deaths. Community transmissions were reported. These cases originated from the provinces of Ituri (31 cases), North Kivi (28 cases), Haut Uele (5) and Tshopo (1). In addition, 15 deaths were recorded in the Ebola Treatment Centers (ETCs).
- A total of 506 new samples were analyzed; 65 of these samples tested positive for the virus.

Bundibugyo, representing a 12.8% positivity rate

ACCUMULATIVE CAS	ACCUMULATIVE DEATH CONFIRMED	LETHALITY	PATIENTS IN ISOLATION/CTE	ACCUMULATIVE HEALED	TRACKING RATES CONTACTS (Of the Day)
9277	4 478	48,3 %	924	2 486	78,0%

Affected provinces and affected health zones

7 Provinces affected

Uele, Ituri, North Kivu, South Kivu, Tshopo, Lower Uele and Sud Ubangi

65 Health Zones affected out of 167 (38,9%)

Summary of key points from the last 24 hours

As of October 14, 2026, 65 new confirmed cases of viral disease Ebola (EVD) has been reported in four provinces: **Ituri (31 cases), North Kivu (28 cases), Haut-Uélé (5 cases) and Tshopo (1).**

In addition, **15** deaths were recorded in the CTEs (6 in Ituri, 6 in North Kivu, 2 in Tshopo and 1 in Haut Uélé), bringing the total number of confirmed deaths for the period in question to 37.

Furthermore, **38 patients (18 in Ituri, 15 in North Kivu and 5 in Haut Uélé)** were declared cured after obtaining two negative results in the control test for Ebola virus disease.

Since the start of the epidemic, the DRC has accumulated **9,277 confirmed cases and 4,478 deaths,** representing a case fatality rate of **48.3** %. Ituri remains the epicenter of the epidemic, accounting for **73.4% of cumulative confirmed cases and 47.7% of new confirmed cases.** reported in the last 24 hours.

65 out of 167 health zones (38.9%), spread across seven provinces, have been affected since the start of the epidemic.

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Table 1. Distribution of confirmed cases and deaths by affected province, as of October 14, 2026.

Province	New cases confirmed (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	31	6806	3 148	46,3%	28/36 (77,8 %)
North Kivu	28	1991	1 133	56,9%	17/34 (50,0 %)
Haut-Uélé	5	398	167	42,0%	7/13 (53,8 %)
Boss	1	64	24	37,5%	7/23 (30,4 %)
South Kivu	0	3	1	33,3%	1/34 (2,9 %)
Bas-Uélé	0	13	4	30,8%	4/11 (36,4%)
Sud Ubangi	0	2		50,0%	1/16 (6,3%)
Total	**65**	**9 277**	**1 4 478**	**48,3%**	**65/167 (38,9%)**

Table 2. Distribution of confirmed cases and deaths by province and zone as of October 14, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Cas (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	**6806**	**3148**	**46,3%**	**31**	**11**	**6**	**17**
Give	11	1	9,1%				0
Understand	9	6	66,7%				0
Ariwara	8	2	25,0%				0
Aungba	12	5	41,7%				0
Bamboo	216	48	22,2%				0
have	2	2	100,0%				0
Bunia	1866	621	33,3%	7	4	1	5
Ladies	40	9	22,5%				0
Drodro	15	7	46,7%				0
Fireworks	84	31	36,9%				0
Gethy	11	7	63,6%				0
Flounder	2	0	0,0%				0
Kilo	38	12	31,6%				0
Team	185	112	60,5%			1	1
Liter	307	155	50,5%	2	0		0
Logo	14	8	57,1%				0
Fight	36	9	25,0%				0
distributed	4	1	25,0%				0
Washing	62	23	37,1%	2	1	0	1
Darkness	277	142	51,3%	8	3		3
Mangala	372	162	43,5%				0
Mongbwalu	713	342	48,0%				0
Nanny	329	188	57,1%			2	2

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Nizi	822	295	35,9%	3	0	2	2
Legumes	134	39	29,1%	1	0	0	0
Jungle	19	5	26,3%				0
Rwampara	1142	398	34,9%	8	3	0	3
Tchomia	76	25	32,9%				0
A valves	THAT	493	THAT				0
North Kivu	**1991**	**1133**	**56,9%**	**28**	**10**	**6**	**16**
Me	491	326	66,4%	9	4		4
Farm	23	7	30,4%				0
Butembo	307	187	60,9%	7	1	1	2
Ten	1	0	0,0%				0
Kalunguta	74	29	39,2%	1	1		1
Cut	633	389	61,5%	1	1	3	4
Source	3	2	66,7%				0
Kyondo	40	21	52,5%				0
Lubero	29	13	44,8%				0
Be anxious.	69	28	40,6%	3	1	1	2
Alimbong	5	3	60,0%				0
Manguredjipa 15		5	33,3%				0
Masereka	39	12	30,8%	2			0
The museums	151	71	47,0%	4	2		2
Mutwanga	24	5	20,8%	1			0
Oicha	47	21	44,7%			1	1
Vuh	40	14	35,0%				0
Upper Uele	**398**	**167**	**42,0%**	**5**	**1**	**1**	**2**
Government Mangbetu	44	19	43,2%				0
Dumb	1	1	100,0%				0
Gombari	3	1	33,3%				0
Mathematics	105	46	43,8%	1	0		0
Lining	101	50	49,5%				0
Beans	5	3	60,0%	0	0	1	1
Ordinary	139	47	33,8%	4	1		1
Boss	**64**	**24**	**37,5%**	1	0	2	2
Bafwasende 10		1	10,0%				0
Kabondo	8	1	12,5%	1	0	0	0
Lubunga	1	0	0,0%				0
Maquis - Kisangani	32	14	43,8%			2	2
Mangobo	10	7	70,0%				0
Boss	2	0	0,0%				0
Wanie - Arugula	1	1	100,0%				0
South Kivu	**3**	**1**	**33,3%**	0	0	0	0
Miti -Murhesa 3		1	33,3%				0
Bas Uélé	**13**	**4**	**30,8%**	0	0	0	0

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Blind	1	1	100,0%				0
Viadana	6	1	16,7%				0
Claim	1	0	0,0%				0
Walk	5	2	40,0%				0
Sud Ubangi 2		**1**	**50,0%**	0	0	0	0
Hair	2	1	50,0%				0
Total	**9277**	**4478**	**48,3%**	**65**	**22**	**15**	**37**

***These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.**

NA : Not Applicable.

Table 3. Status of alerts notified as of October 14, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	164	2	166	15	3	113	0	18	8
Bas Uélé	151	2	153	27	2	123	0	29	0
Ituri	1 487	85	1 572	147	82	1 340	1	229	86
North Kivu	1 854	66	1 920	252	66	1 602	0	318	148
Sud Ubangi	57	0	57	2	0	55	0	2	0
Boss	176	4	180	12	4	142	0	16	0
Total General	**3 889**	**159 4 048**	**455**		**157**	**3 375**	**1**	**612**	**242**

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

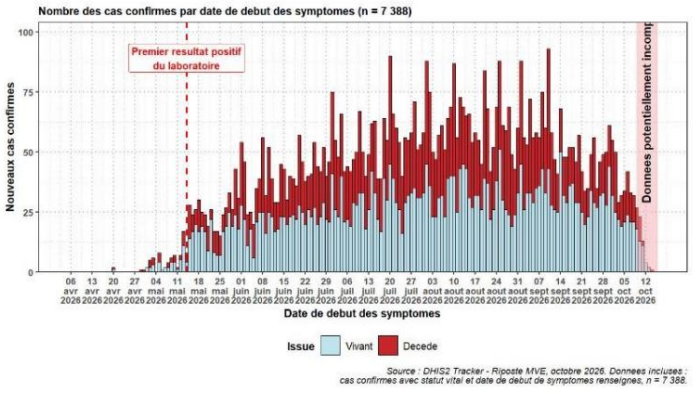
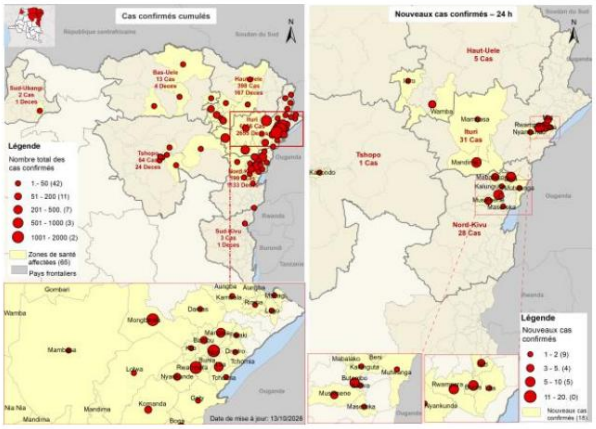


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



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1.COORDINATION

1.1. Main Actions

• Support for response activities in the affected provinces through the technical pillars.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

As of October 14, 2026, 4,048 alerts were reported in the six provinces that submitted their data (Ituri, North Kivu, Haut-Uélé, Bas-Uélé, Tshopo, and Sud-Ubangi). Of these, 3,988 (98.5%) were verified, including 612 (15.3%) validated as suspected cases, all of which were investigated (100), but only 242 (39.5%) were transferred to treatment centers/COVID-19 treatment centers.

Of the 33,264 contacts to be followed up in the six provinces that submitted their data, 25,956 were seen in the last 24 hours, representing a follow-up rate of 78.0%, down from the previous day (81.8%) and below the required threshold of 95.0%. This rate was 100.0% (44/44) in South Ubangi, 95.2% (596/626) in Bas-Uélé, 85.0% (880/1,035) in Tshopo, 84.0% (1,362/1,622) in Haut-Uélé, where the Rungu health zone (149 contacts) did not report, 79.1% (13,680/17,303) in North Kivu, where 1,727 contacts were never seen, including 1,495 in Beni (86.6%), and 74.4% (9,394/12,634) in Ituri. Only South Ubangi and Bas-Uélé reached the 95% threshold.

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

On October 14, two (2) alerts were notified to the PoE/PoC points, both in North Kivu: two bodies were intercepted at the Foner Kasindi PoC (Mutwanga Health Zone), originating from Masambo and Kasindi, and were validated and referred to the Kasindi Health Center for testing. In Ituri, the PoE/PoC pillar recorded its 67th positive case, reported on October 11 at the new Biakato Mayi PoC (Biakato Health Association, Mandima Health Zone); in Tshopo, tents replaced parasols at the ONATRA PoC thanks to a donation from the National Humanitarian Food Programme (PNHF), and two alerts (PK 15 on the Banalia road and PK 23 on National Route 4) remain pending. In total, 284,157 travelers passed through the PoE/PoC points in the five provinces.

Table 4. Status of key PoE/PoC indicators by province, as of October 14, 2026

Indicators	Ituri	North Kivu	South Kivu	Boss	Haut-Uélé	Global
Report completeness	96,7% (59/61)	95,2% (20/21)	55,6% (5/9)	100,0% (18/18)	90,0% (9/10)	93,3% (111/119)
% of functional strategic PoE/PoC	100%	100%	ND	100%	90%	ND
Name of people switched to PoE/PoC	139 818	92 616	8 211	21 275	22 237	284 157
% of travelers screened	97,9%	98,2%	98,7%	98,5%	99,3%	98,2%
% of travelers who washed their hands	97,0%	98,2%	98,7%	98,3%	99,4%	97,7%
% of travelers made aware	99,3%	98,2%	100,0%	100,0%	100,0%	99,1%
Number of alerts notified	0	2	0	0	0	2
Number of live alerts validated (suspected cases)	0	0	0	0	0	0
Number of bodies recovered today	0	2	0	0	0	2
% of suspected cases transferred to the CTE/CT	THAT	THAT	THAT	THAT	THAT	THAT
% of lifeless bodies swabed/secured	THAT	100,0%	THAT	THAT	THAT	100,0%
% of suspected cases investigated within 4 h	THAT	100,0%	THAT	THAT	THAT	100,0%

2.2.2. Challenges

In Ituri, the poor encoding of PoE/PoC alerts in the DHIS2 Tracker, insufficient mobility for supervision in the health zones of Rwampara, Nizi, and Bunia, and delays in supplying inputs to some PoCs persist; the Avakubi PoC remains non-functional due to insecurity. In North Kivu, the completeness is

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95.2% (20/21), the Butuhe Point of Contact (PoC) remains non-functional since the security incident of September 26, 2026. The security threat persists against service providers along the Butembo axis. Alerts from the PoCs are not being addressed promptly, and the PoCs lack consumables, communication equipment, and a dedicated ambulance. In Haut-Uélé, the Bombombi and Barrière Cadenas PoCs did not report any incidents. In Bas-Uélé, only 19 out of 41 sites are functional (46.3%), no Points of Emergency (PoE) have been reinforced, PNHF services are not operational, and the PoCs lack water, transportation, and connectivity.

2.3. Laboratory

2.3.1. Main actions

- **Ituri: 31 new positive results** (18 living and 13 deceased) out of 244 samples received and analyzed **(positivity: 12.7%).**
- **North Kivu: 28 new positive results** (18 living and 10 dead) out of 216 samples received and analyzed **(positivity: 13.0%).**
- **Haut-Uélé: 5 new positive results** (4 living and 1 death) **out of 23 samples received and analyzed (positivity: 21.7%).**
- **Tshopo: 1 new confirmed case (live) out of 8 samples received and analyzed (positive: 12,5%).**
- **Bas Uélé:** 15 new samples received at the Buta laboratory, all analyzed negative.

2.4. Infection Prevention and Control (IPC/EDS)

2.4.1. Main actions

- **Ituri:** 20 out of 36 rings were opened (55.6%), 37 out of 45 households were decontaminated (82.2%), and 5 out of 6 health facilities were decontaminated (83.3%). Of 94 EDS alerts (including 2 postponements from the previous day), 61 EDS were carried out (64.9%), and 64 out of 81 bodies were swabed (79.0%).
- **Haut-Uélé:** 2 out of 2 rings were opened (100%), 5 out of 5 households were decontaminated (100%), and 3 out of 3 ESS (100%) were completed in Isiro, Rungu, and Pawa; 3 ESS were assessed (45.0% at Notre-Dame de Neisu Hospital, 29.0% at the CS, and 25.8% at the Bafwabaka Hospital), and 53 PPLs (People Living with Distress) were briefed (10 in Isiro, 20 in Pawa, 18 in Wamba, and 5 in Boma Mangbetu). Of 9 EDS (Emergency Disposal) alerts, 6 EDS were carried out (66.7%), and 7 bodies were swabed without failure. 2 EDS remain in progress in Isiro due to a lack of graves and coffins.
- Bas-Uélé: Service providers from 12 community health centers (CHCs) were monitored in the health zones of Buta, Viadana, Ganga, Bondo, and Koteli. Seventy-two service providers and 28 hygiene workers from 13 CHCs were briefed on the use of personal protective equipment (PPE), waste management, and sorting. The medical waste management systems (MWSs) at Buta General Referral Hospital (HGR) (12/12), Triangle Health Center, Mobenge Health Center, and Bomea Health Center are operational. The household and CHC for the Dakwa case remain to be decontaminated (0/1) due to a lack of equipment. Two health assessments (HASs) were conducted without swabs, with two HAS gaps, one of which was in Buta.

2.4.2. Challenges

- **In Ituri,** the opening of rings remains insufficient (20/36; 55.6%), ECUMER coverage is low, the number of teams is insufficient in Mangala and Mandima, means of transport are lacking in the ZS of Kilo, Nia-Nia, Mandima, Damas, Mambasa and Fataki and a strike by service providers for non-payment is affecting the ZS of Bunia.
- **In Haut-Uélé,** only 5 out of 7 health zones (ZS) have submitted their infection prevention and control (IPC) data, and IPC kits are lacking for health establishments and households. In Bas-Uélé, 16 out of 20 priority health establishments lack functional triage, 17 out of 20 lack sufficient water storage for 72 hours, and 20 out of 20 lack mattresses; the low reporting of community death alerts and weak community engagement limit the effectiveness of emergency health services (EHS). In North Kivu, 50 EHS remain pending due to a lack of family consent, and 27 out of 49 deaths (55.1%) occurred in non-dedicated health establishments, increasing the risk of exposure for service providers.

2.5. Holistic care

2.5.1. Main actions

- **In Ituri,** 88 new admissions (all suspected cases, including 1 PPL in Mongbwalu and 5 in advanced stages) were recorded, compared to 75 discharges, including 18 recoveries, 47 non-cases, and 10 deaths (5 of which were confirmed). Thus, 407 patients (134 confirmed and 273 suspected) are hospitalized, representing an occupancy rate of 40.0% (53.0% for suspected cases and 25.0% for confirmed cases), including 34 critically ill patients; 29 patients were evacuated by ambulance, and 389 out of 398 patients (97.7%) received a hot meal.
- **In North Kivu,** 74 new admissions (8 confirmed and 66 suspected) were recorded, compared to 88 discharges, including 15 recoveries, 63 non-cases, 9 deaths (6 confirmed and 3 suspected), and 1 discharge without medical advice in Beni. Thus,

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453 patients (158 confirmed and 295 suspected, including 65 children under 5 years old) are hospitalized: 330 in CTE/CT for 392 beds, representing an occupancy rate of 84.2%, and 123 in non-dedicated ESS; 7 patients are in critical condition (Beni CTE and Kitatumba CT) and all 19 ambulance trips were successful.

- **In Haut-Uélé,** 16 new admissions (7 confirmed, including 5 in Wamba, and 9 suspected, including 1 PPL) were recorded, compared to 18 discharges: 5 recoveries (4 in Wamba and 1 in Isiro), 10 non-cases, and 3 deaths of confirmed cases (Isiro, Boma Mangbetu, and Rungu). Thus, 43 patients (16 confirmed and 27 suspected, including 5 children under 5) are hospitalized out of 169 beds, representing an occupancy rate of 25.4%, with 1 suspected case in critical condition in Pawa; 43 patients, 55 caregivers, and 70 PPLs received hot meals, and 7 patients were transported by ambulance.

- **In Tshopo,** four new admissions (one confirmed case, two suspected cases from Bafwasende, and the readmission of a recovered patient) were recorded, compared to five discharges: two non-cases, one suspected case who escaped from the Météo treatment center (negative result), and two deaths, one confirmed case at the treatment center and one suspected case at the Bafwasende treatment center. Thus, 13 patients (five confirmed and eight suspected cases) are hospitalized out of 36 beds, representing an occupancy rate of 36.1%, with no patients in serious condition; three confirmed cases remain unaccounted for (two in Bafwasende and the motorcyclist from AS Marie Antoinette). Since the beginning of the program, 484 people have received the experimental treatment, including 41 confirmed cases, and 443 have received post-exposure prophylaxis.

- **In Bas-Uélé,** no new confirmed cases or admissions were recorded; 17 suspected cases were declared non-cases, with no recoveries or deaths. Six suspected cases remain in isolation (Dakwa 1, Viadana 2, and Ganga 3) along with 2 confirmed cases (Dakwa and Ganga), while 33 suspected cases remain in the community due to a lack of adequate isolation conditions, including 27 new suspected cases today.

2.5.2. Challenges

In Ituri, the low capacity of some CTE/CT (Mambasa, Kilo, Lolwa, Mangala and Biakato), the care of 34 critical patients, the lack of food for 10 patients in the Mandima health zone (CT Mandima and CT Makeke) and the low reporting of ambulance movements (21/33) remain worrying.

In North Kivu, the saturation of suspected beds (164/165; 99.4%), the care of 123 patients (29.9%) outside of CTE/CT facilities, an ambulance response capacity limited to 50.0% (14/28) and the incompleteness of ambulance data (Kalunguta) remain concerning.

In Haut-Uélé, poor network coverage in some health zones, demotivation among some staff, insufficient nutritional inputs, and inadequate water and electricity supplies to some treatment centers remain challenges to be addressed.

In Tshopo, the lack of resuscitation equipment at the CTE and of staff for post-exposure prophylaxis weakens the care provided.

In Bas-Uélé, the lack of appropriate nutritional care for isolated patients, of ambulances for the transfer of patients and of standardized CTE, still under construction at the HGR of Buta, remains a major operational challenge.

2.6. Vaccination

2.6.1. Main actions

- **In Ituri,** vaccination of PPLs with the Ervebo vaccine continued: 26 people were vaccinated on October 13, bringing the total to 1,033 vaccinated (435 women and 598 men, including 11 pregnant women and 26 breastfeeding women; 2 MAPI), and research on the effectiveness of the Ervebo vaccine against the BDBV strain continued.
- **In Haut-Uélé,** 470 PPLs were vaccinated out of 1,060 pre-registered (44.3%), including 249 women and 221 men; vaccination started in the Pawa health zone (80 doses), 1,000 additional doses were ordered and a case of MAPI was validated as suspected at the Isiro CTE.

2.6.2. Challenges

- **In Ituri,** the target for PPL and TPL remains to be redefined for the calculation of vaccination coverage (1,007 vaccinated) for a provisional target of 1,000).
- In Bas-Uélé, the provincial stock of Ervebo vaccine is out of stock (0 doses), the start-up of the UCCs is not effective and the funds for vaccination are still not available; vaccination coverage remains at 11.4% (1,500/13,202).
- In Bas-Uélé, the implementation of the UCCs is not effective, and vaccines are not available in sufficient quantities. sufficient and vaccination funds are still pending.

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2.7. Continuity of Care

2.7.1. Main actions

- **In North Kivu,** the baseline assessment of the Nyakunde Medical Center revealed the absence of triage respecting the circuit, the absence of PPE and poor waste management, a low availability of blood products, outdated and under-equipped emergency services (only one oxygen concentrator, no vacuum cleaner or pediatric Ambu bag) and a non-standard neonatology; 30 out of 76 staff (39.5%) are trained in IPC and 2 staff have been infected.

- **In Haut-Uélé,** the training of 7 ZS managers on SIMR continued, materials and equipment for UIV sites were distributed to Isiro, Pawa and Boma Mangbetu and a large capacity generator from the WFP, which will power the 2 UCCs, was installed; the monitoring of the 5 PPL hospitalized in the CTEs continued.

2.8. Risk Communication and Community Engagement (RCCE)

2.8.1. Main actions

- **In Ituri,** 21,398 home visits reached 70,408 people; 391 educational talks brought together 7,958 people, 19 mass awareness campaigns reached 1,711 people, 18 community debates involved 528 people, and 13 advocacy efforts engaged 23 authorities and leaders, particularly in the chiefdoms of Babila Babombi and Bandaka in Mandima. A total of 345 community alerts, including 152 deaths, were reported. 53 out of 78 feedback requests (67.9%) and 11 out of 16 incidents (68.8%) were resolved within 48 hours. The CREC facilitated the pre-listing of 163 contacts around 5 confirmed cases, the decontamination of 24 out of 24 households (100%) and one facility, 13 out of 19 EDS (68.4%), the announcement of 7 out of 7 positive results, and the transfer of 20 out of 20 suspected cases to the CTE. The completeness of CREC reports is only 35.7% (10/28 ZS).

- **In Bas-Uélé,** 91 home visits reached 454 people (Likati, Dakwa, and Ganga health zones), and two mass awareness campaigns reached 1,694 people, including over 750 individuals and 150 schoolchildren at the Limba market (Viadana). 255 students and teachers were sensitized to school-based surveillance in Titule, and 10 teachers from the Anzange Primary School (Bambesa, Ganga) participated in an educational discussion. All eight community alerts, including two fatalities, and the six feedback received were addressed.

- **In North Kivu,** 16,954 home visits reached 62,558 people, 247 educational talks reached 4,140 people, 79 mass communications reached 8,280 people and 6 briefings reached 106 people; 561 community alerts were notified out of 650 expected (86.3%), 31 feedbacks out of 39 (79.5%) and 3 incidents out of 13 (23.1%) were resolved within 48 hours and 2,637 people out of 4,750 (55.5%) were engaged in community actions; 100 members of resistance groups in the Beni SZ were brought together in a reflection session.

- **In Haut-Uélé,** 77 home visits reached 308 people and 5 mass awareness campaigns reached 1,077 students
  Wamba, 4 educational talks 45 people in Isiro (AS Tely and Tuluba) and 30 journalists from 7 community radio stations in Isiro were briefed on MVE, PCI, EDS and SBC;

2.9. Mental Health and Psychosocial Support (MHPSS)

2.9.1. Main actions

- **In Ituri,** 18 out of 41 confirmed cases (43.9%), 98 out of 116 confirmed cases (84.5%), 43 out of 87 new suspected cases (49.4%), and 117 out of 186 suspected cases (62.9%) received SMSPS interventions. The teams supported 264 caregivers and 98 people living with HIV, monitored 26 out of 27 recoveries (96.3%) in the community and 15 out of 15 children in daycare, provided support to 12 out of 16 separated and orphaned households, reintegrated 9 out of 18 recoveries, and reported 47 results, 18 of which were positive; 67 children are currently being monitored.

- **In North Kivu,** 20 of the last 25 confirmed cases (80.0%), 212 confirmed cases under follow-up, 148 new suspected cases, and 215 suspected cases under follow-up received SMSPS interventions in 8 out of 9 health zones that reported (88.9%); teams assisted 88 households and 32 out of 50 households with separated and orphaned children (64.0%), supported 45 people living with HIV, monitored 20 out of 36 children in the community and 4 out of 4 children in daycare, reintegrated 9 recovered individuals, monitored 5 out of 10 recovered individuals (50.0%) in the community, and announced 97 results, including 25 positive ones; 321 people received psychoeducation with UNICEF support

- **In Haut-Uélé,** 3 new confirmed cases, 15 confirmed cases under follow-up, 3 new suspected cases and 19 suspected cases under follow-up benefited from SMSPS interventions; the teams supported 62 caregivers and 63 PPLs, including 25 members of the national coordination, followed 13 children separated in the community, supported 3 EDS and announced 19 results, including 4 positive ones.

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- **In Tshopo,** 5 out of 5 confirmed cases under follow-up (100%), 2 out of 2 new suspects and 5 out of 5 suspects under follow-up benefited from SMSPS interventions in 5 out of 7 health zones that notified (71.4%); 232 people benefited from psychoeducation, 8 PPLs were supported, 4 results, including 3 positive ones, were announced and 1 out of 1 resistance was broken.

2.9.2. Challenges

- **In Ituri,** only 14 out of 28 affected health zones are covered (50.0%), the Mandima health zone, which concentrates 32% of the cases notified today, is not covered and inter-pillar collaboration needs to be strengthened for interventions around cases in Bunia and Mambasa.

- **In Bas-Uélé,** SMSPS coverage remains embryonic, with activities having only just begun in the ZS from Viadana.

- **In Haut-Uélé,** the mobility of community health workers remains difficult in Wamba and Boma Mangbetu, nutritional kits are lacking for weaned infants in Wamba and Isiro, the community health worker lacks computer equipment, and data collection remains weak in Boma Mangbetu and Pawa

- **In Tshopo,** there is a need to train the APS (Auxiliary Police Officers) of the Wanie-Rukula Health Zone and an insufficient number of units communication and motorcycles for the movement of the pillar.

2.10. Logistics
2.10.1. Main actions.

Ensuring the mobility of all field teams in the affected provinces; in North Kivu, construction work on the huts at PoC PK5 and the hut and passenger walkway at Beni airport has begun with IOM support; in Haut-Uélé, materials have been packed for the activation of two PoCs (PK18 and Bangwene) in the Wamba health zone; in Tshopo, tents have been set up at the ONATRA PoC; in Bas-Uélé, construction of the CTE at the Buta General Referral Hospital (partitioning of rooms and toilets) and rehabilitation of the DPS offices with WHO support are continuing, surveillance teams have been deployed to Ganga and Viadana and the packing of inputs for the remaining health zones is planned.

2.11. Security

2.12. Main actions

In North Kivu, the IPC teams benefited from security coverage for the decontamination of the dispensary.
Kabuyunda, with 2 schools and 2 households in the Kasanga AS, with a laboratory and 2 households in the AS
Tamende and a household in the Mandrandele health zone (Beni health zone); in Haut-Uélé, the security situation is stable in all 13 health zones, with the exception of a few health zones in Wamba (Betongwe axis), and the insecurity mapping has been updated; in Bas-Uélé, the security situation has been assessed and response activities have been secured, although the construction of guard posts at the sites and mobility remain challenges

2.13. PSEA (Prevention of Sexual Exploitation and Abuse)
2.13.1. Main actions

- **In Ituri,** a two-day briefing on the village-based approach integrating the PSEAH (Protection of Children and Adolescents with Disabilities) and accountability concluded in the Mandima and Mayuwano health districts (Mandima health zone) with 87 community health workers, village chiefs, and community leaders (36 women and 51 men), all of whom signed the code of conduct. A coordination meeting for gender-based violence (GBV) activities was held with the Provincial Gender Division in Bunia. In Haut-Uélé, 7 health workers from the Isiro Community Health Center, 16 pregnant women attending antenatal care at the VDO (Veterinary and Obstetric Care Center) (Mayogo health district), and 10 members of the Boma Mangbetu Community Health Zone (ECZS) were sensitized on the PSEAH and the reporting process using the toll-free numbers 495555 and 151. The ECZS members signed the code of conduct.

- **In North Kivu,** 15 providers from the Beni clinic for the cured (8 women and 7 men) were briefed on PSEA and accountability and signed the code of good conduct; 1,161 people were sensitized on PSEA and the toll-free numbers 151 and 495555 in the health zones of Katwa (842 people in the Mabwe and Masuli health areas), Butembo (209 people) and Beni (110 people in the Ndindi, Ngongolio and Tamende health areas).

2.13.2. Challenges

At the national level, 3 out of 7 health districts (42.9%), Ituri, Haut-Uélé, and North Kivu, reported their PSE (Public Health Support and Education) activities to the national coordination, with only one supporting partner reporting (WHO). Coverage of affected areas remains low, visibility tools are insufficient, some partners are not aligned, and the effectiveness of community health centers is low. The proportion of health zones aware of the reporting mechanism remains low in Ituri (41.7%; 15/36), Haut-Uélé (46.2%; 6/13), and North Kivu (17.6%; 6/34).

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In the past 24 hours, 506 out of 513 suspected cases (98.6%) have been investigated in the provinces that submitted their data.
However, 26 suspected cases remain uninvestigated in Ituri. Furthermore, the contact tracing rate has decreased from 81.8% the previous day to 78.0%, although it remains above the required threshold of 95.0%. This situation necessitates strengthening epidemiological surveillance and contact tracing to limit the risk of community transmission.
Maintain a level of vigilance regarding epidemiological surveillance aspects, consolidate IPC and CREC actions at the community level while providing quality care in dedicated care structures with adequate reception capacities.

TIMELINE OF KEY EVENTS

ÿ From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.

ÿ Between May 17 and 18, 2026, national authorities , with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.

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